



Mental health consumers who decline assessment or follow up

This procedure should be read in conjunction with the South Metropolitan Health Service (SMHS) Mental Health [Triage Assessment of Referrals in Community Mental Health](#)

1. Background

1.1. About this document

This procedure provides guidance for mental health clinicians within the SMHS where a consumer declines assessment or follow-up by a site or service.

1.2. Key definitions

Term	Definition
Capacity	A person has capacity if they are capable of: • understanding the information being given, the nature of the decision and its consequences • retaining the information as necessary to make a decision • using and weighing the information in the decision-making process • communicating their decision in some way.
Consent to (medical) treatment	In the context of health care, consent is a patient's agreement that a health practitioner can proceed to perform a specific proposed treatment.
Multidisciplinary team (MDT)	A team of mental health staff usually comprised from a range of disciplines including psychiatry, psychology, nursing, occupational therapy, social work, pharmacy, peer workers, welfare officers, Aboriginal Mental Health Liaison and other relevant groups who collaborate in the assessment, planning and delivery of care.
Personal Support Person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, their nominated person; the parent or guardian of a child or any enduring

guardian of an adult [MHA 2014](#) s.7. Some consumers may use the term kin to refer to a family member.

2. Procedure

2.1. Referrals

Persons may be referred to a mental health service by different sources, including general practitioners, other health services, community support services, Western Australian (WA) Police, carers or personal support persons or by self-referral.

A person who has been referred to a mental health service has the right to decline assessment or clinical follow-up. Where a person referred to a SMHS mental health service declines initial mental health assessment or clinical follow-up the mental health service should determine the potential impact to the person or others from not engaging with assessment or appropriate follow up care. This will be determined in consultation with the referrer and any involved personal support persons and take appropriate action.

Actions may include:

- seeking further information from the referrer
- seeking collateral information from involved carer or personal support person
- for Aboriginal consumers, cultural factors such as disconnection from Country, kinship obligations, grief and loss, and historical trauma should be considered as part of the risk assessment.
- engage Aboriginal Mental Health Liaison to assist with culturally appropriate engagement and support, particularly where the consumer has declined assessment or follow-up.
- seeking collateral information from other appropriate services e.g. WA police
- requesting a mental health assessment in the community by the community treatment team or relevant Mental Health Crisis Centre / Assessment and Treatment Team after-hours
- consider whether criteria for referral for examination by a psychiatrist have been met
- notification must be made to WA Police Force where there is a genuine and serious concern for the consumers welfare and safety, or they are at risk of harming others.

Refer to:

- [Community Mental Health Status Assessments: Role of Mental Health Clinicians Policy](#).

The multidisciplinary team will discuss and determine the appropriate course of action according to the consumers assessed level of need. Refer to [Appendix A](#) Flow Chart to the guide clinical decision-making process.

The mental health service will notify the patient's general practitioner or other referrer that they have been unable to contact or follow up the consumer.

Staff will notify clinical incidents that have or could have resulted in harm, and which are attributable to health care provision (or lack of) rather than the consumers underlying condition in the Datix Clinical Incident Management System.

2.2. Consent

Referral to a mental health service does not indicate consumer consent to assessment or follow up. Adults are presumed to have capacity in the context of a decision to be made unless shown otherwise. Children do not have capacity unless they are a mature minor. Clinicians must seek consent from a voluntary consumer before providing psychiatric treatment except in circumstances of emergency psychiatric treatment. Consent must be given freely and voluntarily.

For Aboriginal consumers, clinicians should ensure that consent discussions are culturally safe and inclusive. Where appropriate, with the consumers consent Aboriginal staff or interpreters should be involved to support understanding and informed decision-making.

Refer to:

[WA Health Consent to Treatment Policy](#)

2.3. Documentation

Clinicians who take actions to locate, assess and / or treat a consumer who has declined assessment or follow-up, must document the identified needs and risk assessment of the consumer and actions taken and rationale in the consumers medical record and Psychiatric Services Online Information System (PSOLIS).

Where relevant, documentation should include any cultural considerations discussed, involvement of Aboriginal support staff, and efforts made to provide culturally safe care.

3. Compliance Monitoring

Each SMHS site or Service Director/Co-Director is to ensure compliance with this procedure via clinical incident reporting and monitoring.

4. Legislation

There are no legislative obligations relating to this procedure for which a statutory penalty is imposed for a breach.

- [Mental Health Act 2014](#)

5. Related Documents

5.1. Department of Health policies

- [MP 0175/22 Consent to Treatment Policy](#)
- [MP 0122/19 Clinical Incident Management Policy](#)
- [MP 0099/18 Community Mental Health Status Assessments: Role of Mental Health Clinicians Policy.](#)
- [WA Aboriginal Health and Wellbeing Framework 2015-2030](#)

5.2. SMHS policies

- [Triage Assessment of Referrals in Community Mental Health](#)

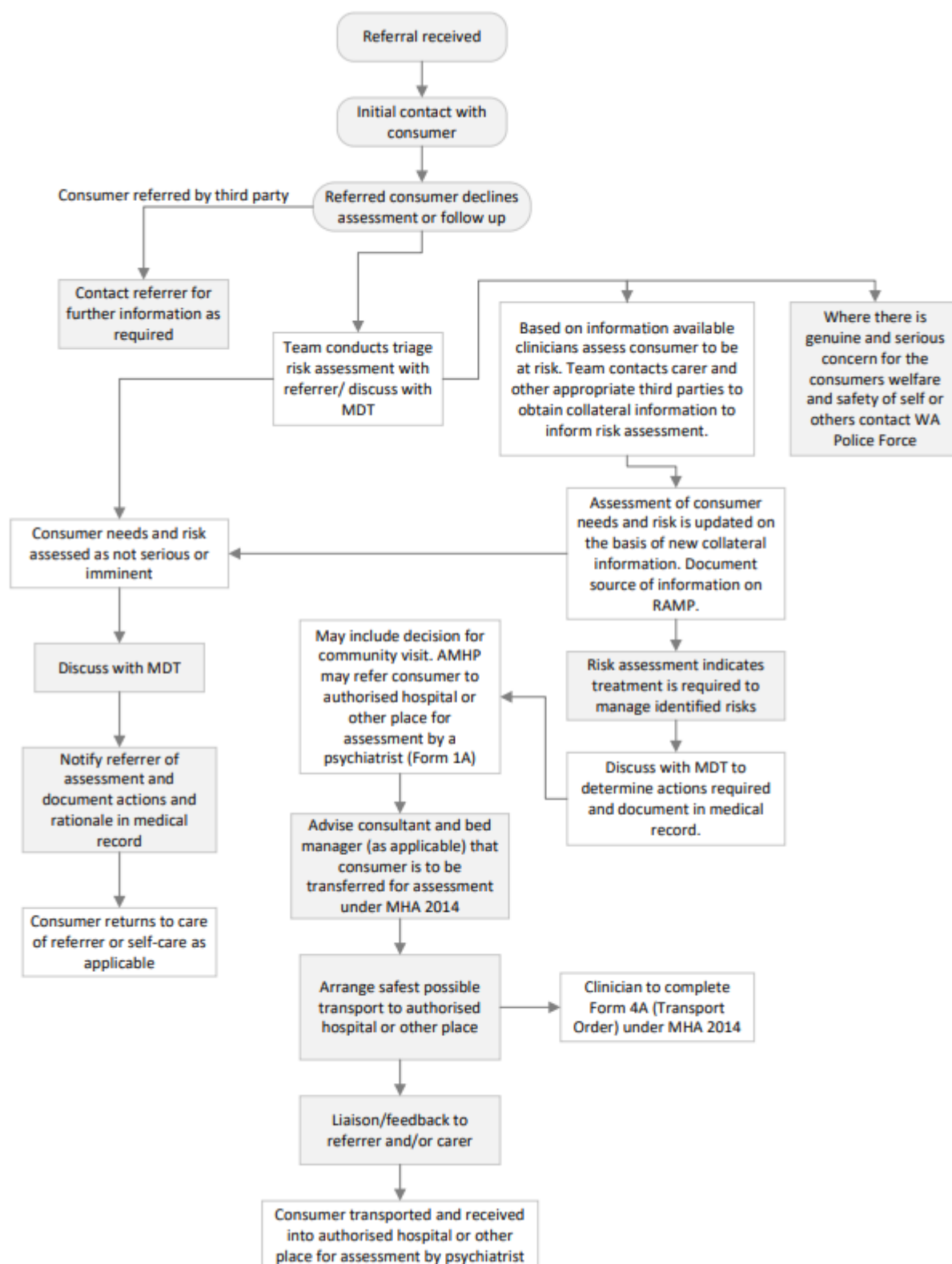
5.3. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP. V3	5 December 2025	14 November 2025	12 November 2028	November 2028	Triennial Review

5.4. Approval

Approval by	Executive Director, Mental Health
Approval date	2 December 2025

6. Appendix A: Consumers who decline assessment



This document can be made available in alternative formats on request.

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